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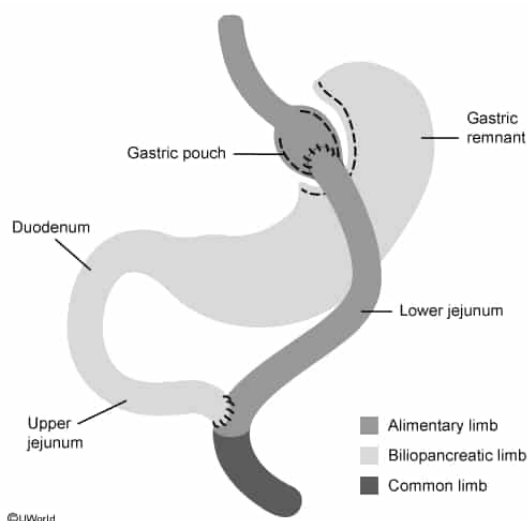


Figure 1 RYGB surgically divides the digestive tract into the alimentary limb and the biliopancreatic limb, which then merge into the common limb.

RYGB facilitates weight loss by reducing the functional volume of the stomach and altering intestinal anatomy to induce nutrient malabsorption. Although the gastric pouch has the same overall function as a normal stomach, the procedure is associated with several complications, such as steatorrhea (fatty stools) due to disruption of lipid digestion, and a more alkaline environment in the gastric pouch. Animal studies also suggest that RYGB promotes the growth of weight loss-inducing gut bacteria. Researchers have found that transplanting gut microbiota of obese postoperative RYGB mice into non-operated obese mice lacking gut flora may reproduce the therapeutic effects of RYGB in transplant-recipient mice.

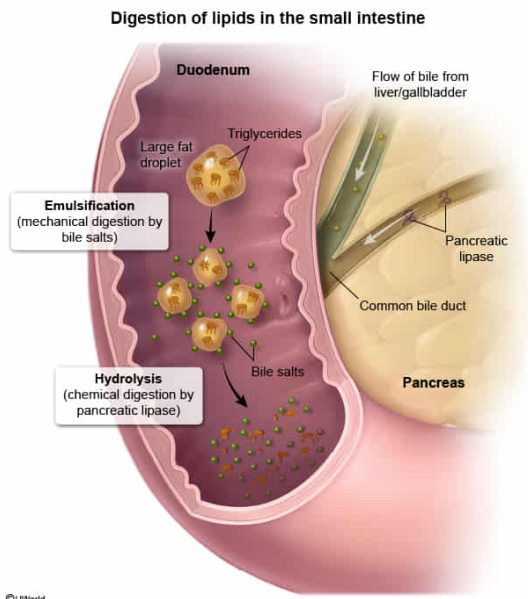
Which organ synthesizes a compound that directly facilitates the mechanical digestion of lipids?

- ☐ A. Pancreas [15%]
- ☐ B. Salivary glands [22%]
- ☐ C. Gallbladder [16%]
- ✓ ☒ D. Liver [45%]

Correct

44% answered correctly

Explanation:



The **liver synthesizes bile**, a *nonenzymatic* solution stored in the gallbladder and released into the small intestine to aid in the mechanical digestion of lipids. Bile is composed of bile salts, bile pigments (eg, bilirubin), and cholesterol. **Bile salts** act as detergents to solubilize fats during digestion; they possess both a hydrophobic region that associates with the surface of lipids and a hydrophilic region that associates with water. Consequently, bile salts break down large lipid globules into smaller droplets (micelles) in a process known as **emulsification**.

Emulsification is **an example of mechanical digestion**, a nonenzymatic process that *physically breaks down* food particles into smaller pieces. Additional examples of mechanical digestion include chewing in the mouth and churning in the stomach due to peristaltic waves.

(Choice A) Chemical digestion is carried out by acids and enzymes and involves the cleavage of chemical bonds within macronutrients to form simpler compounds that can be absorbed. Emulsified lipids have increased surface area on which pancreatic lipase can exert its enzymatic effect (ie, chemical digestion of lipids). As a result, lipase secreted from the pancreas facilitates the chemical digestion (enzymatic hydrolysis), *not* mechanical digestion, of triglycerides within micelles into free fatty acids and monoglycerides.

(Choice B) The salivary glands are **exocrine glands** in the mouth that secrete saliva, a lubricating fluid containing the enzymes lipase and amylase. Lingual lipase facilitates the chemical, *not* mechanical, digestion of lipids. In contrast, salivary amylase hydrolyzes (chemically digests) the polysaccharide starch into the disaccharide maltose.

(Choice C) The gallbladder synthesizes *no digestive compounds* and serves only as a reservoir (storage site) for bile produced by the liver.

Educational objective:

Bile is synthesized in the liver, stored in the gallbladder, and released into the duodenum to aid lipid digestion. Bile salts, a key component of bile, mechanically digest lipid globules by physically breaking them down into smaller droplets in a process known as emulsification.

Subsequently, pancreatic lipase chemically digests emulsified lipids.

Time spent:0 sec

Subject:
Biology

QID:400870

System:
Digestion and Excretion

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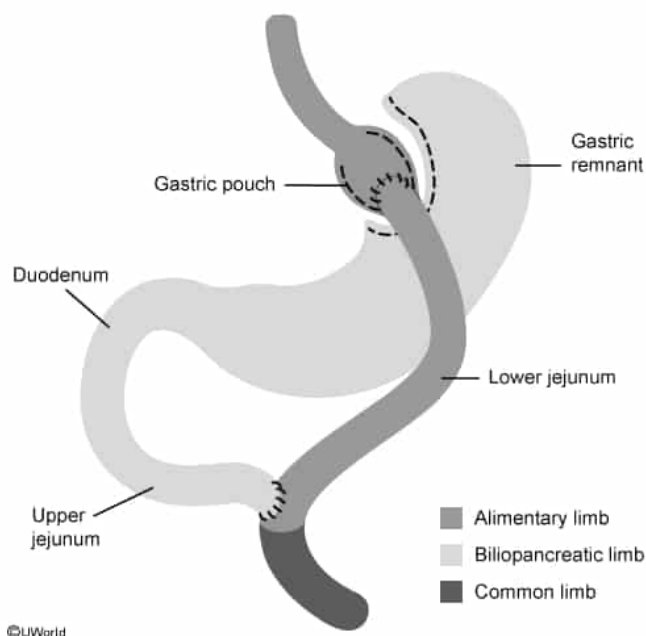
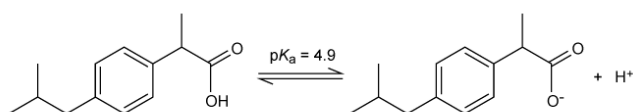


Figure 1 RYGB surgically divides the digestive tract into the alimentary limb and the biliopancreatic limb, which then merge into the common limb.

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stools) due to disruption of lipid digestion, and a more alkaline environment in the gastric pouch. Animal studies also suggest that RYGB promotes the growth of weight loss–inducing gut bacteria. Researchers have found that transplanting gut microbiota of obese postoperative RYGB mice into non-operated obese mice lacking gut flora may reproduce the therapeutic effects of RYGB in transplant-recipient mice.

Nonsteroidal anti-inflammatory drugs (NSAIDs) are weakly acidic compounds that normally pass through the acidic environment of the stomach in a non-ionized form until they reach the small intestine, where they become ionized in the less acidic lumen. Ionized NSAIDs, shown below, are more soluble and therefore dissolve faster.



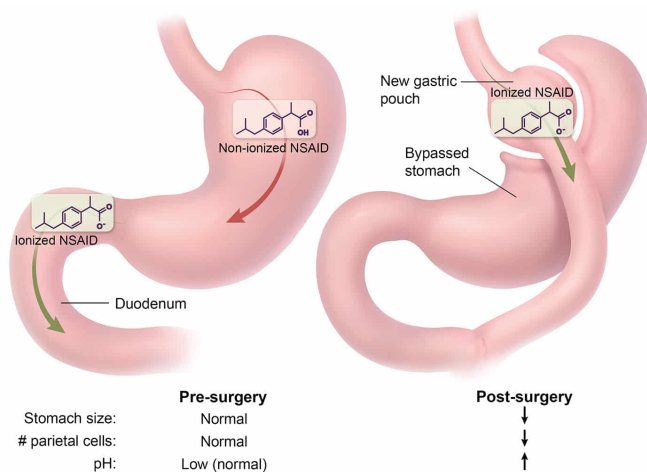
Would NSAID solubility in the gastric pouch of postoperative RYGB patients be higher than normal?

- ☐ A. No, because RYGB decreases the pH of gastric juice [5%]
- ☐ B. No, because RYGB increases the pH of gastric juice [20%]
- ✓ ☒ C. Yes, because RYGB increases the pH of gastric juice [66%]
- ☐ D. Yes, because RYGB decreases the pH of gastric juice [7%]

Correct

66% answered correctly

Explanation:



Gastric juice is a digestive fluid primarily composed of **hydrochloric acid** (HCl), which is secreted by **parietal cells** in the stomach. Gastric juice is a highly acidic solution that maintains the stomach lumen at a low pH of 1-3; this acidic environment is required for protein digestion (via activation of proteolytic enzymes) and to kill harmful bacteria. After sufficient churning, the stomach expels a semifluid mass of partially digested food mixed with gastric juice, known as **chyme**. The highly acidic chyme passes into the duodenum, where it is **neutralized** by the bicarbonate ions secreted from the pancreas via the pancreatic duct and the alkaline bile released from storage in the gallbladder. As a result, the pH of the chyme is increased to ~6.

According to the question, NSAIDs are normally non-ionized in the acidic stomach and ionized in the less acidic small intestine. Therefore, NSAIDs generally ionize in less acidic (ie, more alkaline) environments. However, the passage states that RYGB results in a more alkaline environment in the gastric pouch. This is likely because the surgery creates a smaller stomach (ie, gastric pouch) with a *decreased* internal surface area compared with the presurgery stomach.

As a result of the surgery, the **small gastric pouch** formed in **RYGB** likely contains significantly **fewer parietal cells** than a normal-sized stomach. Consequently, the total amount of HCl secreted by parietal cells is likely decreased, thereby increasing the pH of gastric juice in the lumen of the gastric pouch. Due to the **increased pH (decreased acidity) of the gastric juice**, NSAIDs ionize more readily and exhibit increased solubility in the gastric pouch.

(Choices A and D) The pH of gastric juice is increased, *not* decreased, post-RYGB. As a result, NSAID solubility is increased in the gastric pouch because RYGB increases the pH of gastric juice.

(Choice B) Due to the increased pH of gastric juice of the post-surgery stomach, NSAID solubility *is* higher than normal.

Educational objective:

The acidic pH of the stomach is maintained by gastric juice, which is composed primarily of hydrochloric acid. This acidic environment is required for protein digestion and to kill harmful bacteria. When gastric juice mixed with food (chyme) enters the duodenum, it is neutralized by bicarbonate ions (produced in the pancreas) and bile (released from storage in the gallbladder).

Time spent:0 sec

Subject:

Biology

QID:400871

System:

Digestion and Excretion

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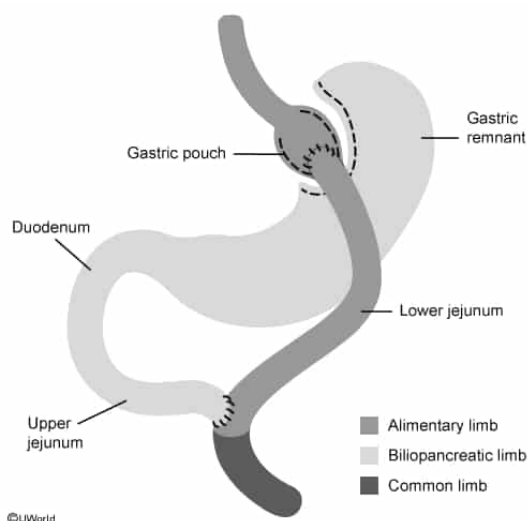


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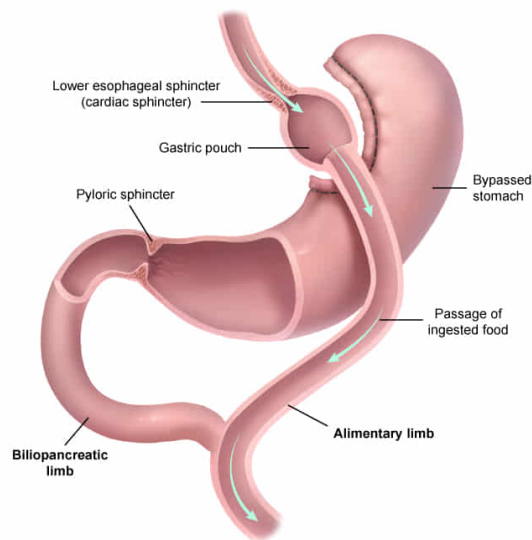
In RYGB patients, ingested food will NOT pass through the:

- ☐ A. cardiac sphincter. [16%]
- ☐ B. intestinal villi. [8%]
- ✓ ☒ C. pyloric sphincter. [66%]
- ☐ D. cecum. [7%]

Correct

66% answered correctly

Explanation:



Food is chewed in the mouth and swallowed as a bolus into the **gastrointestinal tract**, a long tube comprising the esophagus, stomach, small intestine, and large intestine. At certain sections, **sphincters**, or **rings of muscle**, divide the tract into segments with distinct functions. A sphincter is generally constricted so that its center is closed but relaxes to allow the natural passage of substances based on physiological requirements.

The segments of the small intestine include the duodenum, jejunum, and ileum. The **pyloric sphincter** is a muscular ring located at the junction of the stomach and the duodenum; this sphincter controls the flow of chyme, or partially digested food, from the stomach into the duodenum. In **RYGB patients**, ingested food passes through the alimentary limb: Nutrients flow from the small gastric pouch into the surgically attached jejunum, **bypassing the pyloric sphincter** in the biliopancreatic limb.

(Choice A) The cardiac sphincter (ie, lower esophageal sphincter) is a muscular ring located at the junction of the esophagus and the stomach that controls the passage of food into the stomach. According to Figure 1, the cardiac sphincter is unaffected after surgery, so ingested food passes *normally* through this structure.

(Choice B) Intestinal villi are small, finger-like projections found on the epithelial lining of the small intestine that increase its surface area for nutrient absorption. The duodenum is involved in nutrient digestion, and the jejunum and ileum are involved in nutrient absorption. In RYGB patients, ingested nutrients are digested and absorbed through the intestinal villi of the jejunum in the alimentary limb.

(Choice D) The **large intestine** is composed of the cecum, colon, and rectum. The cecum, the first segment of the large intestine, is a small pouch connected to the appendix. Ingested food normally passes from the ileum into the large intestine because RYGB does not alter large intestine anatomy.

Educational objective:

At certain sections along the gastrointestinal tract, sphincters, or rings of muscle, divide the tract

into segments with distinct functions. Ingested food passes from the esophagus into the stomach through the cardiac sphincter, and from the stomach into the duodenum through the pyloric sphincter.

Time spent:0 sec

Subject:
Biology

QID:400872

System:
Digestion and Excretion

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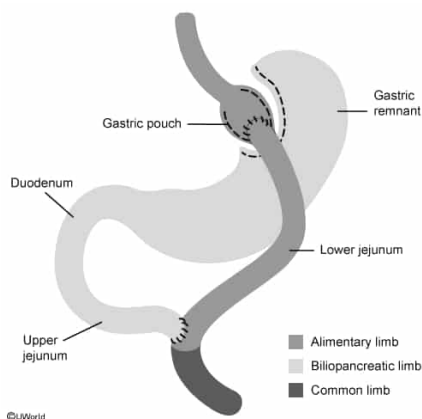
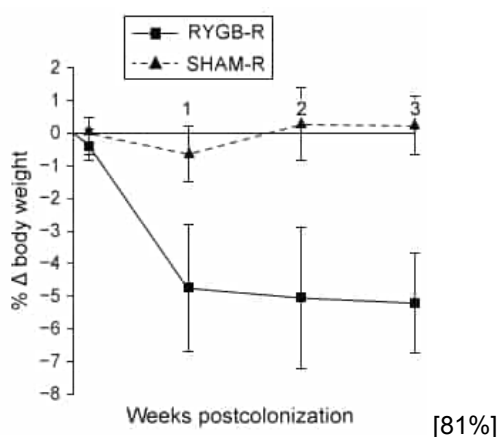


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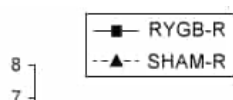
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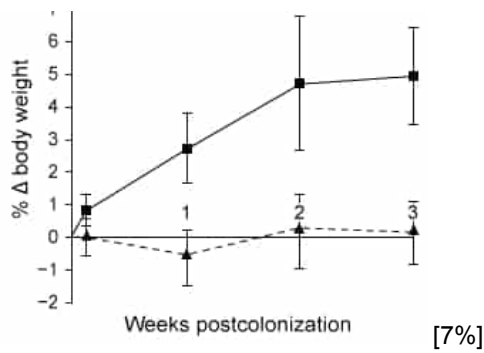
Obese mice underwent either RYGB or sham (placebo) surgery and two weeks post-surgery, their fecal matter was transplanted into the guts of non-operated obese mice lacking gut flora (germ-free recipients). The body weight of RYGB flora recipients (RYGB-R) and sham surgery flora recipients (SHAM-R) was measured for 3 weeks postcolonization. Which graphic shows the expected result of this experiment?

✓ ☒ A.

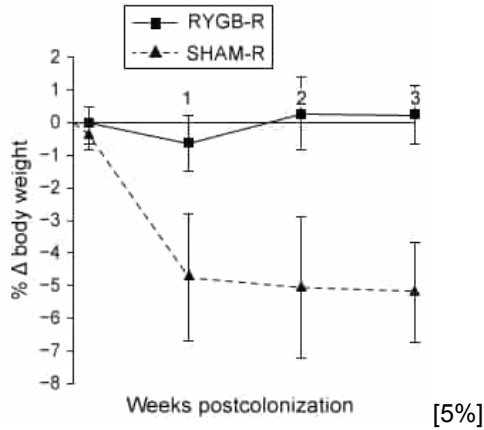


☐ B.

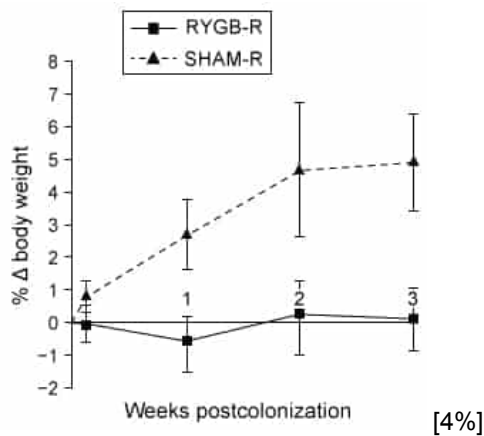




☐ C.



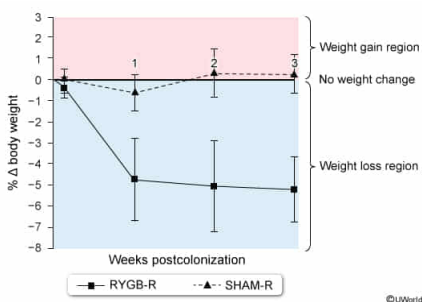
☐ D.



Correct

82% answered correctly

Explanation:



According to the passage, RYGB induces weight loss by reducing the functional capacity of the stomach and promoting the growth of weight loss-inducing bacteria in the gut (as shown in animal studies). The passage states that **weight loss**, the **therapeutic effect of RYGB**, is transmissible when the gut microbiota of RYGB-treated mice are transplanted into recipient mice lacking these bacteria.

In the given experiment, non-operated obese mice lacking gut flora (ie, germ-free recipients) were transplanted with fecal (bacteria-rich) samples from mice that had either undergone RYGB or sham surgery. The passage study showed that when fecal samples from postoperative RYGB mice were transplanted into germ-free mice, the RYGB transplant-recipient mice **(RYGB-R) were expected to lose weight**. In contrast, when fecal samples from postoperative sham mice were transplanted into germ-free mice, these transplant-recipient animals **(SHAM-R) were expected to show no significant percent change in body weight**. The graph in Choice A is the only option that depicts these expected findings.

(Choice B) Although the SHAM-R group in this graph shows the expected result of no significant change in body weight, the RYGB-R group shows a positive percent change in body weight (ie, weight gain) instead of the expected weight *loss*.

(Choice C) This graph shows the *opposite* of the expected results in both groups: The RYGB-R group shows no change in weight, and the SHAM-R group shows substantial weight loss.

(Choice D) This graph shows that the RYGB-R group (expected to show *weight loss*) has no significant change in body weight whereas the SHAM-R group (expected to show *no weight change*) experiences significant weight gain.

Educational objective:

Research suggests that restructuring the composition of gut flora can influence body weight. The gut generally contains nonpathogenic bacterial flora that symbiotically cohabit with enterocytes. These microorganisms produce vitamins (eg, vitamin K, necessary for clotting) and aid with nutrient and drug metabolism.

Time spent:0 sec
Subject:
Biology

QID:400873
System:
Digestion and Excretion

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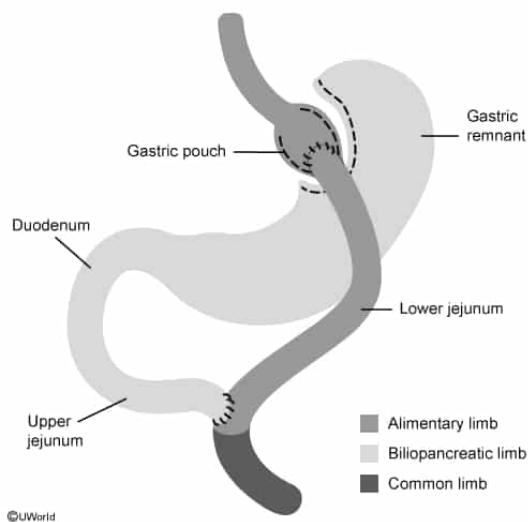


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All the following are secreted by cells in the alimentary limb of RYGB patients EXCEPT:

- ☐ A. pepsinogen. [14%]
- ✓ ☒ B. trypsinogen. [35%]
- ☐ C. intrinsic factor. [25%]
- ☐ D. mucus. [23%]

Correct

35% answered correctly

Explanation:

Stomach secretions		
Stomach cell type	Secretory product	Function of secretory product
G cells	Gastrin	Signals parietal cells to secrete hydrochloric acid
Parietal cells	Hydrochloric acid	Primary component of gastric juice; activates proteolytic enzymes, kills microbes & denatures (unfolds) proteins
	Intrinsic factor	Aids in the absorption of vitamin B ₁₂ in the ileum
Chief cells	Pepsinogen	Zymogen (inactive form) of pepsin, which cleaves polypeptides into smaller peptides when activated by the low pH of gastric juice
	Gastric lipase	Carries out hydrolysis of lipids in the stomach
Mucous cells	Mucus & bicarbonate	Protect the stomach walls from autodigestion by gastric juice, which contains acid & proteases

The **stomach** generally functions to churn, digest, and store food. The alimentary limb formed during RYGB consists of the gastric pouch (ie, surgically reduced stomach) and the attached lower jejunum (ie, a portion of the small intestine). The passage states that in RYGB patients, the **smaller gastric pouch has the same overall function as a normal-sized stomach**, and therefore would secrete the following:

- The zymogen pepsinogen (inactive) is released by gastric chief cells and converted into pepsin (active form) on mixing with hydrochloric acid (HCl) in the gastric juice. Pepsin then digests proteins in the stomach **(Choice A)**.
- Intrinsic factor, a glycoprotein released by gastric parietal cells, is crucial for proper absorption of vitamin B₁₂ **(Choice C)**.
- Mucus is released by mucous cells in the stomach to form a **protective** physical barrier against autodigestion by HCl and proteolytic enzymes in the gastric juice. Bicarbonate ions released from stomach epithelial cells form a chemical buffer barrier under the mucus barrier **(Choice D)**.

The pancreas generally secretes proteolytic enzymes (eg, trypsinogen, chymotrypsinogen) into the pancreatic duct, which empties into the duodenum to aid digestion of chyme in the lumen. However, in RYGB patients the duodenum is located in the biliopancreatic limb. Accordingly, pancreatic trypsinogen is released from the pancreas and secreted into the duodenum within the **biliopancreatic limb**, *not the alimentary limb*.

Educational objective:

The cells in the stomach secrete the following: intrinsic factor (from parietal cells), which aids in the absorption of vitamin B₁₂; pepsinogen (from chief cells), which cleaves polypeptides when

activated by HCl; and mucus (from mucous cells) and bicarbonate (from epithelial cells), which protect the stomach lining against autodigestion by gastric juice.

Time spent:0 sec

Subject:
Biology

QID:400874

System:
Digestion and Excretion

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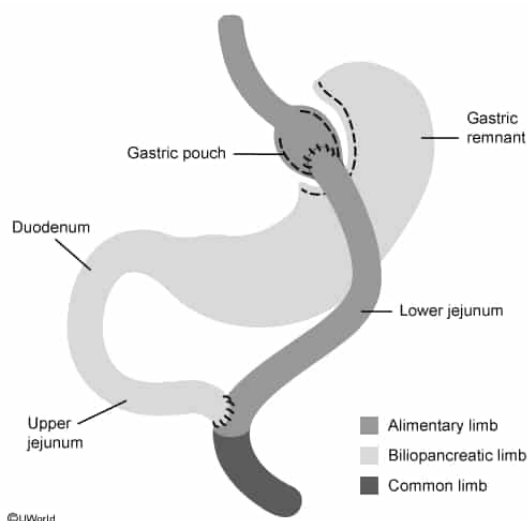


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A staple line leak is an unanticipated opening of surgical staples at an incision. In RYGB, a staple line leak at the junction of the biliopancreatic limb and the alimentary limb would cause gastrointestinal tract contents to enter:

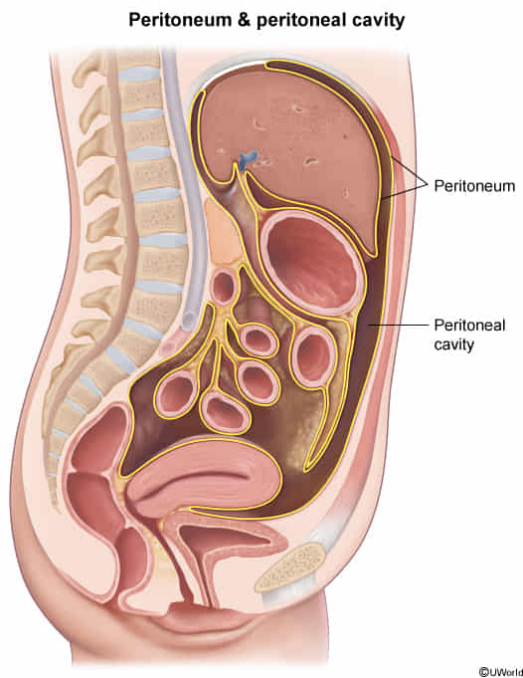
- ☐ A. the perineum. [5%]
- ☐ B. the peritoneum. [20%]
- ☒ C. the peritoneal cavity. [68%]

☐ D. the pleural cavity. [5%]

Correct

69% answered correctly

Explanation:



The **peritoneum** comprises two tissue membranes that line the abdomen: the parietal layer, which lines the abdominal wall, and the visceral layer, which covers the abdominal organs. The **peritoneal cavity** is a potential space (ie, an area between two adjacent structures that may press together) between the parietal and visceral layers of the peritoneum. The peritoneal cavity is found within the abdomen and contains organs such as the liver, stomach, and intestines. Based on the given scenario, a **staple line leak at the junction of the biliopancreatic limb and the alimentary limb** would **allow gastrointestinal (GI) tract contents to enter the peritoneal cavity**.

(Choice A) Contents of the GI tract would not leak into the perineum because the perineum is the area between the anus and the scrotum (in males) or vulva (in females).

(Choice B) The peritoneum is a tissue membrane, and therefore not a space/cavity into which GI contents can enter.

(Choice D) The **pleural cavity** is a thin, fluid-filled space between membranes of the lungs and would not be affected by a staple line leak in the postoperative RYGB GI tract.

Educational objective:

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Time spent: 0 sec

Subject:
Biology

QID: 400875

System:
Digestion and Excretion